

SMS Medical College & Hospital Jaipur

Department of Surgical Gastroenterology

Pawan

Flak Biliary Pomerotele

6.	7/9/19	- no fast cough - vital stable <u>PR</u>	Dr. SYED KHALID AKHTAR Senior Resident Department of Gastroenterology S.M.S. Hospital, Jaipur	
7.	8/9/19	- no fast cough - vital stable - PR jet	Dr. SYED KHALID AKHTAR Senior Resident Department of Gastroenterology S.M.S. Hospital, Jaipur	
8.	<u>12/09</u> <u>POD#1</u>	<u>close B G team</u> - no fast cough - pr vital stable <u>PR</u> - spironometry - mobilization	RT :- 40ml (B) D/o :- 150ml (SS) U-o :- 3 times <u>PR</u>	M-at D/o
9.	<u>13/09</u>	<u>close on G team</u> - no fast cough - vital stable	<u>PR</u> <u>D/S</u>	
10.				

BC-A-1

adav

Investigation Chart

HCV:-
HIV:-
HbsAg :-

[illegible]

Whitford

6/27	7/27
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Rs:-

S.M.S. MEDICAL COLLEGE & S.M.S. HOSPITAL, JAIPUR (RAJASTHAN)

ADMISSION & DISCHARGE RECORD

BED HEAD TICKET (BHT)

MMAAY

110325245330424

HID No.: 110325245330424

IPD No.: A/SMH/25/112707 Date: 06/09/2025 11:22



Name: Pawan Kumar Sharma Father's Name: Omprakash Sharma
 Spouse Name: Mamta Devi Age/Sex: 43Y /M Mobile No: 9887313392
 Category: MMAAY Amount: 0 Scheme/TID: MAA_YOJNA
 Department/Unit/Ward: GASTROINTESTINAL SURGERY/ UNIT 1/ 5 A WARD
 MLC Type: Brought By: Brought From:
 Address: Hospital Road, Ward No 12, Surajgarh, Jhunjhunu Surajgarh, - 333029, RAJASTHAN

Package Code MMAAY	186756039C	MMAAY TID No.	
Type of Admission →	Routine (R)/ Emergency (E)/MLC (M)	Date of Discharge	Total Hospital Days (Stay)
Provision Admission diagnosis	Flare Bilary Pancreatitis	ICD Code	
Final Diagnosis (Write Principal Diagnosis First)		Date & Time of Expiry	
Operative Procedure		Date	Time
Result (Please tick)	Discharge	LAMA	Expired
If expired	Autopsy Done	Autopsy not done	
Cause of Death	T06092541802719		
Name & Signature of Duty Doctor with seal	Name & Signature of Unit Head with seal	Name & Signature of Nursing Staff	
IF MEDICO-LEGAL CASE (MLC) (FILLED BY FORENSIC MEDICINE)			
Seen by Dr. of For. Med. Department Remarks →		Date	Time
MLC No.		Name & Signature of Dr. of Fore. Med.	
If referred from Outside then details		Date	Time
Clearance of MLC cases, if fit for Discharge/Any Other Remarks		Name & Signature of Dr. of Fore. Med.	
MEDICAL RECORD DEPARTMENT			
Received By		Date	
Checked by		Sheets	

Note: BHT should be deposited in MMAAY Department Immediately, After discharge & within 24 hrs. after death.
 Phone No. 0141-2518222/2518422, Website :- www.medicaleducation.rajasthan.gov.in

S.M.S. MEDICAL COLLEGE & S.M.S. HOSPITAL, JAIPUR (RAJ.)

CONSENT FORM/सहमति प्रपत्र

(To be filled by patient or his or her relative/रोगी अथवा उसके संबंधी द्वारा भरे जाने के लिये)

(1) FOR TREATMENT

Concerned Doctor & Hospital Staff have completely appraised me for myself/my above relative about diagnostic examination or tests/Biopsy/Blood transfusion/Surgery/Medical treatment/Caner treatment/Drug to be used for Anesthesia/All expenses etc. & I/my relative agree for the same. I give complete consent in my full sense from myself/my above relative without any condition/pressure for these. Any Doctor & Hospital Staff will not be responsible for any mishap.

(1) उपचार हेतु

संबंधित चिकित्सक एवं चिकित्सालय स्टाफ ने मुझे मेरे/मेरे उपरोक्त सम्बन्धी के नैदानिक परीक्षण अथवा जाँच/बायोप्सी/रक्त आधान/शल्य चिकित्सा/भेषज उपचार/कैन्सर उपचार/चेतना-शून्य के लिये उपयोग में ली जाने वाली औषधी/समस्त खर्च इत्यादि के बारे में समझा दिया है एवं इनके लिये मैं/मेरे सम्बन्धी राजी है। मैं अपने पूर्ण होश-हवास में बिना किसी शर्त/दबाव के इनके लिये अपनी/अपने सम्बन्धी की पूर्ण सहमति देता हूँ। किसी भी अप्रिय घटना के लिये कोई भी चिकित्सक एवं चिकित्सालय स्टाफ जिम्मेदार नहीं होंगे।

हस्ताक्षर/Signature :

नाम / Name :

रोगी से सम्बन्ध / Relationship with patient :

मोबाईल या दूरभाष नम्बर / Mobile or Telephone Number :

दिनांक / Date :

समय / Time :

(2) FOR LEFT HOSPITAL AGAINST MEDICAL ADVICE (LAMA)

I am leaving/taking away myself/my above relative from the Hospital against the concerned Doctor's Advice, on my responsibility. Concerned Doctor & Hospital Staff have appraised me about consequences of the same. Concerned Doctors & Hospital Staff will not be responsible for any adverse outcome including death of the patient etc.

(2) चिकित्सा राय के विरुद्ध चिकित्सालय छोड़ने हेतु (चि.रा.वि.चि.छो.)

मैं, स्वयं/मेरे उपरोक्त सम्बन्धी को अपनी जिम्मेदारी पर संबंधित चिकित्सक की राय के विरुद्ध चिकित्सालय से जा/ले जा रहा हूँ। संबंधित चिकित्सक /चिकित्सालय स्टाफ ने मुझे इसके पश्चात् पड़ने वाले दुष्प्रभाव के बारे में बता दिया है। इससे संबंधित किसी भी प्रकार के दुष्प्रभाव, मरीज की मृत्यु इत्यादि के लिये संबंधित चिकित्सक, चिकित्सालय एवं चिकित्सालय स्टाफ जिम्मेदार नहीं होंगे।

हस्ताक्षर/Signature :

नाम / Name :

रोगी से सम्बन्ध / Relationship with patient :

मोबाईल या दूरभाष नम्बर / Mobile or Telephone Number :

दिनांक / Date :

समय / Time :

DEATH REPORT

This is to certify that there was no negligence on the part of Medical & Para-Medical Staff and all due care was taken to save the life of the Patient.

Name & Signature of Staff on Duty

Name & Signature of M.O. on Duty

Name & Signature of Unit Head

Signature of Nursing Superintendent

Signature of Medical Superintendent

IF PATIENT ALLOWED PAYING WARD

Allotment

Account Settled

Allotted Ward No. :

Amount (Rupees) :

Date :

Time :

Receipt No. :

Date :

Patient having privilege certificate attached

Signature of Allotment officer/Staff

Signature of ward I/o or Cashier

S.M.S. MEDICAL COLLEGE & S.M.S. HOSPITAL, JAIPUR (RAJASTHAN)
EMERGENCY/OUT PATIENT DEPARTMENT (OPD) ADMISSION SHEET

Patient's Name → Father's Name	Pawan Kumar Sharma	Sex	M	Age	43y
Advised Admission in Department →	Surgical Gastroenterology	Date & Time Admission	01/09/25 ; 11:00am		
Type of Admission →	Routine (R) / Emergency (E) / MLC (M)				
Reffer from →					

Provisional Diagnosis :-

Ans : 6/40 open Neurotomy for Biliary

Presenting Complaints :-

pancreatitis & the start in situ
(01.07.25)

Rdx
- RI
- RAC

Admit ↓ Dr. Ran Daga in
unit I ward 50/01

Vitals :-

B.P. if req.		Pulse if req.		Respiratory Rate if req.	
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General Physical Examination :-

Systemic Examination :-

Investigation Advised :-

Treatment Advised :-

डॉ. निरंजन जागिड़
सहायक प्राध्यापक
सurgical Gastroenterology
S.M.S. Medical College & Hospital
Jaipur

Dr. Shuman

Name & Signature of Duty Doctor with Seal

S.M.S. MEDICAL COLLEGE & S.M.S. HOSPITAL, JAIPUR (RAJASTHAN)
HISTORY AND PHYSICAL EXAMINATION SHEET

Patient's Name	Pawan,	Sex/Age		C.R. No.	
		Date of Admission	06/05/14	Adm. No.	
Unit	2	Ward	LA	Bed No.	

Sudden onset pain abdomen (upper) | 5 months back
 Vomiting bilious | Radiating to back
 No h/o hematemesis
 No h/o melena
 Non-alcoholic
 Non-smoker
Febrile
 Then admitted in SDMH Jaipur on 01/04/15
 USG WIA (01/04/15) - Bulky pancreas with Acute
 Cholecystitis
 MRCP (02/04/15) - Bulky pancreas & peripancreatic
 stranding, inflammation & stranding, S10
 Severe Acute Pancreatitis & cholecystitis
 (N) CBD
 CT WIA (08/04/15) - Bulky edematous pancreas & 50% necrotic pancreas
 marked peripancreatic inflammation & mild peripancreatic
 stranding, severe Acute (sub) fluid
 Necrotizing pancreas (10/10)
 USG guided cholecystostomy on (09/04/15) → Removed (18/8/25)
 PCD - USG guided 15/04/15 - multiple
 CT WIA (07/08/25), large WON in pancreas extending lesser sac & infrarenal
 peripancreatic region & residual pancreatic
 marked attenuation of (SMV) & (prox. spl. vein) &
 Collaterals

Name & Signature of Doctor with Seal

S.M.S. MEDICAL COLLEGE & S.M.S. HOSPITAL, JAIPUR (RAJASTHAN)
HISTORY AND PHYSICAL EXAMINATION SHEET

Patient's Name	Sex/Age	C.R. No.
	Date of Admission	Adm. No.
Unit	Ward	Bed No.

Presenting Complaints :-

Surgey - (09/05/24) - Open Neurosurgery & large drainage

- ✓ 300gm neoplasm removed & 1 litre purulent collection
- ✓ Bulge in mesocolon
- ✓ Body & Tail Neoplasm

Present History :-

Cholecystomy : Output gradually increased

meep (06/06/25)

Panc. duct is not dilated

Past History (Family/Drug/Any Operative History) :-

mild to mod. Acidosis.

CD (N)

Erept + DPT (2stent) R

020/07/25

General Physical Examination :-



Ac. Biliary pancreatitis

Systemic Examination :-

Respiratory :-

CVS :-

GIT :-

CNS :-

Provisional Diagnosis :-

Name & Signature of Doctor with Seal

S.M.S. MEDICAL COLLEGE & S.M.S. HOSPITAL, JAIPUR (RAJASTHAN) **DOCTOR'S ORDERS**

Initial all orders, cancel by crossing through and initialing, Re-write all orders when turning over and after major Operations.

Patient's Name		Sex/Age		C.R. No.	
		Date of Admission		Adm. No.	
Unit		Ward		Bed No.	

	Day →				
	Date →				
	Blood Pressure (mm of Hg) →				
	Pulse (Per minutes) →				
	Temperature (F) →				
	24 Hrs. Input/Output				
	Diet →				
	Investigations →				

S. No.	Medicines ↓				
	Post-op Orders : 11/9/25				
	- Propped up 30°				
	- NPO x 6 hrs	NS	1-1-1		
	- IVF NS 2 120ml/hr	DS	1-0-1		
	DS 5				
	- inj Moroprom 1gm IV TDS		1-1-1		
	- inj Amikacin 750mg IV OD		1-0-0		
	- inj Pantop 40mg IV OD		1-0-0		
	- inj Diclofenac 75mg TDS		1-1-1		
	- inj Tramadol 50mg IV SOS		SOS		

Name & Signature of Doctor with Seal

S.M.S. MEDICAL COLLEGE & S.M.S. HOSPITAL, JAIPUR (RAJASTHAN)
NURSES DAILY RECORD

Patient's Name		Sex/Age		C.R. No.	
		Date of Admission		Adm. No.	
Unit		Ward		Bed No.	

Date ↓ All Instructions to be initiated by persons administering and witnessing

consent for lap cholecystectomy

- हमें हमारे चिकित्सक ने हमारी भाषा में हमें समझा दिया है कि हमारे मरीज के पेट में दर्द उनके पित्त की थैली में पथरी के कारण है जिसका उचित इलाज आपरेशन द्वारा किया जाएगा। इस आपरेशन को दूरबीन पद्धति द्वारा किया जाएगा व आवश्यकता पड़ने पर चीरा लगाकर भी किया जा सकता है। आपरेशन के दौरान अत्यधिक रक्त रिसाव, पित्त की नली में चोट, संक्रमण, दोबारा आपरेशन की आवश्यकता पड़ना आदि खतरों से रक्त हो सकते हैं। यह सब जानने के बाद भी हम इस आपरेशन की अनुमति देते हैं व सभी खतरों की जिम्मेदारी लेते हैं।

हम किसी भी हानि होने पर चिकित्सक को दोषी नहीं ठहराएंगे।

Name & Signature of Nursing Staff/Incharge

HAN)

5786
06-09-25

राजस्थान सरकार
सवाई मानसिंह चिकित्सालय, जयपुर
बाह्य रोगी उपचार पत्र

मूल प्रति

Open No : as0031
Printed On : 06-09-2025 10:35 AM Valid Upto: 15-09-2025 11:59 PM
ID : 1103252-45530424 Visit No: V/SMH/25/2256063
Patient : MR. PAWAN KUMAR SHARMA 43Y / Mobile : 991 9887315392
Father's Name: OMPRakash SHARMA Mother's Name: ANUPurna DEVI
Category/Amount: DOMICILE OF RAJASTHAN/Rs.0.00
Department/Unit: GASTROINTESTINAL SURGERY/UNIT 1
Building/Cabin: DHANVANTRI OPD FORTH FLOOR/GASTROINTESTINAL SURGERY- CABIN
Visit Date : 06-09-2025 Visit Type: RE-VISIT
Operator : RAM.RATAN.CHOUDHARY1 JanAdhar : XXXXXXXXXXXX
OPD Days : Mon.Wed.Sat
Disclaimer - In case of any side effect from medicines, call helpline
number (toll free): 1800-180-3024 (Mon. to Fri. 9.00 AM to
5.30 PM) inform

43/M

H/O Biluig Panwadia & Shweta
Swar (01.03.25)

18/08: 0.19/0.02

~~Amplere~~

Amplere - 41

01/10 Nausea 09.05.25

PCC = 01.04.25

Removal 1st time

Amir & Dr. Ram Raja

Unit 1

band 08/10

डॉ. निशान जांगिड़
सहायक संचालक
सर्जिकल चिकित्सालय, जयपुर
सवाई मानसिंह चिकित्सालय, जयपुर
Dr. Shweta
HR

हस्ताक्षर
चिकित्सक

प्राप्ति
हस्ताक्षर
रोगी

“अंगदान महादान”

मुख्यमन्त्री नि:शुल्क दवा योजना के उपयोग में आने वाली आवश्यक दवाएँ
नि:शुल्क दवा वितरण केन्द्र पर उपलब्ध हैं।

वेबसाईट से घर बैठे ही ऑनलाईन अपॉइंटमेंट एवं जॉच रिपोर्ट प्राप्त करें - <http://ihms.health.rajasthan.gov.in/IHMS/online/index.html>
दवाओं से किसी भी प्रकार के दुष्प्रभाव होने पर हेल्पलाइन नंबर (टोल फ्री) : 1800-180-3024 पर (सोम से शुक 9:00 AM to 5:30 pm) सूचित करें
SMS Enquiry : 0141-2518422-222 | Ambulance-2564222

SAWAI MAN SINGH HOSPITAL JAIPUR

SAWAI RAM SINGH ROAD TONK ROAD

LAB REPORT

Patient's Name : MR. Pawan Kumar Sharma
 Age/Gender : 43Y /M
 Bill Id :-
 Bill No : SMSH/I/25/3903276
 Department : GASTROINTESTINAL SURGERY

HID : 110325245330424
 Bill Date : 01-09-2025
 Patient Category : DOMICILE OF RAJASTHAN
 Report Date/Time : 01-09-2025 10:53:07 AM
 Opd/lpd No : V/SMSH/25/2211868

Test Name/Parameter	Result/Notes	Unit	Reference Range
BIOCHEMISTRY Report			
SERUM UREA	27	mg/dL	10-50
SERUM Creatinine	0.71	mg/dL	Male 0.6-1.5 Female 0.6-1.4
Electrolyte			
Na	139.4	mmol/L	135-145
K	4.16	mmol/L	3.5-5.5
Cl	101.1	mmol/L	95-105
SERUM BILIRUBIN			
Total Bilirubin	0.19	mg/dL	Up To 1.1
Direct Bilirubin	0.02	mg/dL	Up To 0.25
Indirect Bilirubin	0.17	mg/dL	0.2-1.0
SGOT	15.54	U/L	0-35
SGPT	22	U/L	0-40
SERUM AMYLASE	41	U/L	0-96

Hematology Report

COMPLETE BLOOD COUNT (CBC)

Hemoglobin	12.8	gm/dl	12-18
TOTAL WBC	9.55	10 ³ /UL	4-11
TRBC	4.75	10E6/UL	4.5-5.5
MCV	89.7	fl	83-101
MCH	26.9	pg	27-32
MCHC	30.0	g/dl	32-37
RDW-CV	16.6	%	11.5-14.5
PLATELET COUNT	495	10 ³ /UL	150-400
HCT	42.6	%	40-50
PCT	0.41	%	0.15-0.62
MPV	8.3	fl	7-9
Neutro	56.4	%	40-70
Lympho	34.0	%	20-40
Mono	5.8	%	2-10
Eosin	3.2	%	1-6
Baso	0.6	%	0-1
MCV	89.7	fl	83-101

Test Report are subjected to Technical limitations should be correlated clinically.

***** END OF REPORT *****



S.M.S. HOSPITAL, JAIPUR

SONI HOSPITALS CT & MRI CENTRE

Patient Name	Pawan kumar Sharma	Patient ID	545966731
Age	43 yrs	Date	02-Sep-2025
Gender	Male	Ref. Doctor	SMS

CECT SCAN WHOLE ABDOMEN

Stent is seen in situ in CBD.

Mild left pleural effusion with basal consolidation is noted.

Body of pancreas are mildly atrophic with heterogeneous attenuation of pancreas and presence of ill defined thick collection with multiple air foci in body & tail of pancreas extending in peripancreatic region, it is extending in lesser sac, perisplenic region, left paracolic region, left paranephric region and is involving left iliopsoas muscle, size of collection is approx. 60 x 48 x 234 mm, marked peripancreatic fat stranding is noted extending in mesentery - suggestive of acute pancreatitis with residual intrapancreatic and peripancreatic collection.

Mild bilobar pneumobilia is noted.

Gall bladder is contracted with pericholecystic oedema and few air foci in gall bladder lumen -- ? cholelithiasis ? cause.

A tiny 1 mm sized calculus is noted in upper pole calyx of right kidney and lower pole calyx of left kidney, mild dilatation of right pelvicalyceal system and upper ureter is noted, No obvious calcified calculus is seen in course of ureter.

Mild ascites is noted.

Mild thinning of anterior abdominal wall in umbilical region - ? cause.

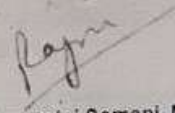
Liver is normal in size and has smooth borders. Hepatic parenchyma shows normal attenuation. Intrahepatic biliary radicals are not dilated. Hepatic vasculature is unremarkable.

Spleen and both adrenals are normal in size and shape.

Urinary bladder is distended and has smooth walls and the rectum appear normal.

Pelvic viscera appears normal.

No paraaortic or aortocaval lymphadenopathy.


Dr. Rajni Somani, M.D.
Rmc: 007217
Consultant Radiologist

This report is not valid for medico-legal purposes • All subject to Jaipur jurisdiction only.

Department of Anaesthesiology, Critical Care & Pain Management

SMS Medical College & Attached Hospitals

JAIPUR-302004 (Rajasthan)

PAC Registration No. 13801 Date 07/09/25 HID No. 30424

PRE-ANAESTHETIC EXAMINATION (PAC)

Name of Patient: Pawan Kumar Age/Sex: 47y/M Weight: _____ Height: _____

Department: _____ Unit: _____ Ward/ICU: _____ BMI: _____

DIAGNOSIS: P/c/o Bilirubin PROCEDURE PLANNED: Cholecystectomy
Pancreatitis

HISTORY:

Cold / Cough / Fever: ⊖

Jaundice / Constipation / Diarrhoea/Vomiting: ⊖

Breathlessness / Chest Pain/ Sweating / Palpitation: ⊖

DM / Hypertension / Obesity (OSA): ⊖

Head Injury / LOC / Convulsions/ Stroke: ⊖

Asthma / Tuberculosis / Congenital Heart Disease: ⊖

Blood transfusion / Bleeding disorder: ⊖

Any Surgical / Anaesthesia history: +

Drug history / Chemotherapy / Radiotherapy: ⊖

h/o Neuro Section C louver
drainage & GA May 25

h/o ERCP on 01/07/25
Indolite

Personal History : Smoking/Alcoholic/ other addiction : ⊖ LMP - _____

H/o Allergy : Nil

Family History : Bleeding disorder / HTN / DM in Family

Case specific history :

Scores : (RCRI - High risk surgery, h/o CAD/CHF / Stroke / DM on Insulin / Creat > 2)

METS : 7.1 RCRI score : _____ out of 6. MACE-Risk : Low / Moderate/High Risk.

GENERAL EXAMINATION

Pulse: 86/hr BP 130/78 SpO₂: _____

RR- 16/hr Temp. AF

Pallor / Icterus / Cyanosis / Clubbing: ⊖

Lymphadenopathy / Edema: ⊖

Psychiatric status:

Calm / Anxious Apprehensive / Agitated / Unstable

AIRWAY EXAMINATION

Inter-incisor gap: 3f

Thromental Distance: 6.5cm

Neck Movt: Ad

Loose / Absent / Artificial teeth: ⊖

Breath holding Time: 725 sec

Anticipated difficult Mask ventilation / Instubation:

Mallamptatii Grade: Gr II

TMJ Movt: Ad

Spine: _____

SYSTEMIC EXAMINATION:

CVS: S, S2P, no murmur

RS: BAE A/E equal, no
adde sound

P/A: Rigid, tender

CNS: Conscious T10/p

(1)

INVESTIGATIONS :

Hb / PCA	13/-	BT/CT		Bl. Sugar-	80	Total S. Bilirubin	
TLC	9.37	PT/INR	14.2/08	HbA1c		Direct / Indirect Bil	
DLC		HIV	MP.	Bl. Urea	25	SGOT/SGPT	16/14
Platelet	476	HBsAg	neg	S. Creat	0.71	Total Protein	
ESR		HCV	neg	Na ⁺ / K ⁺ / Cl ⁻	137/4.5	S. Albumin	
T3 / T4/TSH				Ca ²⁺		A : G Ratio	

Urine Albumin / Sugar

126 = A-m

ECG: NAD

Chest Xray- NAD

2D ECHO-

Case specific investigations if any-

Reference Advised if any-

Adv
 1. Pt is postoperative 1.7 hrs
 Can be taken for & E
 informed & written consent

Accepted in ASA Grade- 1/II/III/IV/V

Any anticipated problem-

PAC INSTRUCTIONS :

1. NBM for :
2. Consent :
3. Blood component required :
4. Any case specific equipments required :
5. Pre Operative medication advice :

RESIDENT DOCTOR
 PAC CLINIC
 JSS HOSPITAL, JALPAIGURI

(NAME AND SIGN OF ANAESTHESIOLOGIST)

REVIEW PAC with date.

निश्चेतन (Anaesthesia) सहमति पत्र

मैं Pawan Kumar Sharma पूरे होश में यह प्रमाणित करता हूँ/करती हूँ।

1. कि मेरे सामने पूरी तरह से मेरी भाषा में वर्णन किया गया है, कि मेरे lap cholecystectomy ऑपरेशन/प्रक्रिया के लिये एनेस्थिसिया सेवाएँ आवश्यक हैं।
2. कि सभी प्रकार के एनेस्थिसिया में कुछ जोखिम है और मेरी प्रक्रिया या उपचार के परिणामों के बारे में कोई गारंटी नहीं दी गई है। एनेस्थिसिया के साथ दुर्लभ ही सही लेकिन जटिलताएँ/समस्याएँ हो सकती हैं और संक्रमण, रक्तस्राव, दाँत का टूटना, दवा दुष्प्रभाव, रक्त थक्का, संवेदना शून्यता, अंग कार्य में कमी लकवा, स्ट्रोक, मस्तिष्क क्षति, दिल के दौरों और मृत्यु की सम्भावना होती है। मुझे समझ है कि जोखिम सभी प्रकार के एनेस्थिसिया पर लागू होती है। यह जानते भी मैं इसकी सहमति प्रदान करता/करती हूँ।
3. कि मेरी प्रक्रिया के लिये प्रयोग ली जाने वाली एनेस्थिसिया प्रक्रिया मेरी शारीरिक अवस्था, मेरी डॉक्टर द्वारा की जाने वाली प्रक्रिया के प्रकार, एनेस्थेसिस्ट की प्राथमिकता और मेरे स्वयं की इच्छा सहित विभिन्न कारकों द्वारा तय होते हैं।
4. कि कभी-कभी सिडेशन के साथ या उसके बिना लोकल एनेस्थिसिया द्वारा इस्तेमाल की जाने वाली एनेस्थिसिया तकनीक पूरी तरह सफल न रहे और इसलिये जनरल एनेस्थिसिया सहित अन्य तकनीक उपयोग करनी पड़ सकती है।
5. कि ऑपरेशन के दौरान, कुछ अदृष्ट परिस्थितियाँ उत्पन्न/प्रकट हो सकती हैं जो हमें आकस्मिक प्रक्रियाएँ करने पर बाध्य करती है, जिनमें मेरी जान बच सके, मैं इसकी सहमति देता हूँ।
6. कि कुछ ना समझ आने पर मैं चिकित्सकों से सवाल पूछ सकता हूँ।
7. कि ऑपरेशन के दौरान, जरूरत पड़ने पर अन्य दवाईयों एवं सम्पूर्ण रक्त या रक्त उत्पाद-लगाने की सहमति/प्रदान करता/करती हूँ। इसकी समस्त जिम्मेदारी मेरी स्वयं उपरोक्त बातें मुझे अच्छी तरह समझ आ गई हैं। मैं इसके लिये सहमति प्रदान करता/करती हूँ। इसकी समस्त जिम्मेदारी मेरी स्वयं की होगी।

रोगी के अँगूठे का निशान/हस्ताक्षर

Anil Sharma

रोगी के रिश्तेदार (Attendent) के अँगूठे का निशान/हस्ताक्षर

Pawan

(11/09/25)

HIGH RISK CONSENT

मैं Pawan Kumar Sharma पूरे होश में यह प्रमाणित करता/करती हूँ कि मेरे सामने मेरी भाषा में मुझे बता दिया है कि मुझे निम्नलिखित कारणों से निश्चेतन प्रक्रिया के दौरान सामान्य परिस्थितियों की अपेक्षा अधिक खतरा है-

- 1.
- 2.
- 3.

इन बीमारी/बीमारियों की वजह से निश्चेतन/शल्य-चिकित्सा के दौरान कई जटिलताएँ उत्पन्न हो सकती है जो मेरे लिये प्राणघातक हो सकती है।

यह सब बातें जानकर तथा समझकर भी एनेस्थिसिया के लिए अपनी सहमति प्रदान करता/करती हूँ। इसकी समस्त जिम्मेदारी मेरी स्वयं की होगी, अस्पताल प्रशासन या चिकित्सा की नहीं।

Attendent

Anil Sharma

(Relation to Patient) Patient

Sign/Thumb impression

Pawan

11/09/25

of Patient

INVESTIGATIONS:

DATE: 11/09/25

REGIONAL / GENERAL ANAESTHESIA / MAC

Patient Identification by:

1. Nursing Staff
2. Surgeon
3. Anaesthesiologist

Preop. status of patient:

NBM Status Complete Consent: /N

Time of induction: 11:45 am

GCS: 15/15

HR: 78

Time of Starting Surgery: 12:05 pm

Monitoring:

ECO₂ ✓ Temp
NIBP ✓ IBP
SPO₂ ✓ CVP
FLO₂ ✓ PA cath
EICO₂ ✓ Urine output
Gas Analysis Ryle's tube

Airway
Mask
SFA
SLT / DLT/other

RESPIRATORY MANAGEMENT

Laryngoscopy:
Direct/FO/RSI
CL Grade: I
Airway adjunct:
Stylet / Bougie

ETT / Tracheostomy:
cuffed / uncuffed
Oral / Nasal
Size: 8.5 mm
Fixed at: 22 cm
Throat pack: Y/N

Ventilation:
Circuit: open / Closed
Spont / Controlled
Mode: VCV
TV/ P/ FiO₂ / S<sub>O₂
RR: 18 PEEP: 3</sub>

INTRAOPERATIVE MONITORING

TIME LINE:

DRUGS and infusions

Premedication:

Induction:

MR

Maintenance:

Inhalation

Oxygen:

N₂O/Air

Relaxant

Analgesic

Inotropes

Other drugs

Inj. Midazolam 1mg IV + Inj. Glycopyrrolate 0.2mg IV
Inj. Fentanyl 100 mcg IV
Inj. Propofol 100mg IV
Inj. Succinylcholine 100mg IV

Sevoflurane 0.2-2.1.

SD 1.

Inj. Atracurium 25mg IV LD + 5mg IV SOS

Inj. Diclofenac 75mg IV + Inj. Paracetamol 1gm IV

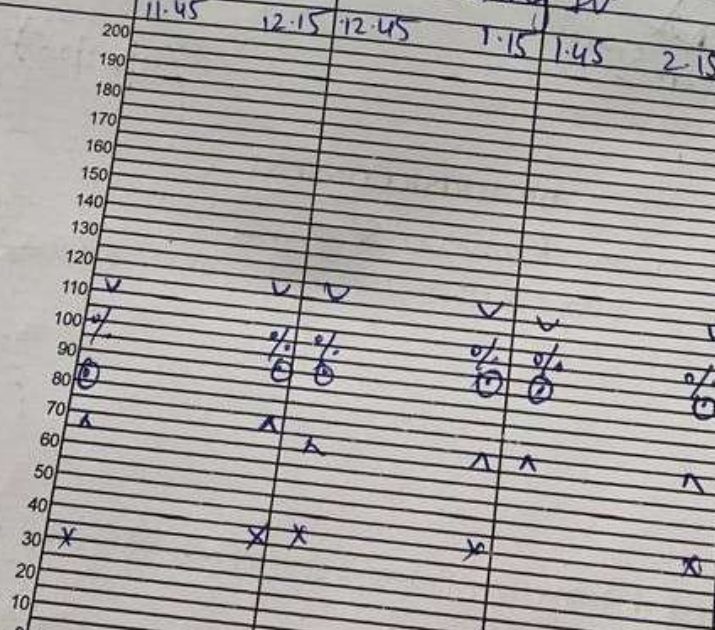
Inj. Ondansetron 4mg IV

VITALS:

HR ⊕
SBP V
DBP Λ
SpO₂ %
EtCO₂ X

P-peak +
CVP *
CO #

11:45 12:15 12:45 1:15 1:45 2:15



RL1

RL2

RL3

IV line-1

IV line-2

Urine:

Blood loss:

Other

G @ time:

/ pCO₂ / pO₂

3 / Na⁺ / K⁺

Intraoperative Event if any:

Name of Anaesthesiologist : Dr. Pooja Kharan
 Name of Surgeon : Dr. Ramesh Dr. Sayaj Mawale
 Procedure : lap cholecystectomy
 Emergency / Elective : Elective

BP : 110/70
 Time at end of surgery :

RR : 12

SpO₂ : 100%

Patient position : supine

Warmers : Y/N

Tourniquet time:

VASCULAR ACCESS:

IV Cannula :

Central line :

Arterial line :

REGIONAL ANAESTHESIA TECHNIQUE:

After completion of surgery, all anaesthetic agents are withdrawn. Airway suctioning done. When spont. resp. occurs, pt is given IV Neostigmine 2.5mg + IV atropine 1mg. When muscle power is achieved, pt is extubated.

TOTAL

Extubation : Y/N
 Conscious level :
 Orientation to T/P/P :

Response to Verbal command
 Muscle power :
 Cough Reflex :

Respiration :
 Color of skin :

POST OPERATIVE INSTRUCTIONS-

I. TRANSFER TO ICU / RECOVERY / WARD

1. Routine Care :
2. Position-NBM for
3. Monitoring Continuous ECG / SPO 2/ BP every 15 minutes
4. Sedation Score/Pain Scores every 15 minutes

II. Oxygen :

III. Analgesia :

IV. I.V. Fluids :

V. Antibiotics :

VI. Other Medications :

VII. VENTILATORY Instructions-

Adv
 1) NPO till further orders
 2) SLO chart
 3) vitals monitoring
 4) inform SOS

POSTOPERATIVE/RECOVERY AREA VITALS...

Time	PR	BP	RR	Spo2	Sedation score	Pain score (VAS)
Shifting Vitals	110	130/90	12	100%		

PACU discharge Criteria (Modified Aldrete Score)

Parameter	STATUS	SCORE
Oxygenation	SPO2 > 92% on room air	2
	SpO2 > 90% on oxygen	1
	SpO2 < 90% on oxygen	0
Respiration	Breathes deeply and coughs freely	2
	Dyspnoeic, shallow or limited breathing	1
	Apnoea	0
Circulation	Blood pressure + 20% of baseline	2
	Blood pressure + 20-50% of baseline	1
	Blood pressure more than + 50% of baseline	0
Consciousness	Fully awake	2
	Arousable on calling	1
	Not responsive	0
Activity	Moves all extremities	2
	Moves two extremities	1
	No movement	0
TOTAL SCORE before shifting from PACU :		
Discharge Time from RECOVERY / PACU :		

Female 0.6-1.4

SERUM UREA 21.9 mg/dL 10-50
BLOOD SUGAR RANDOM 90.2 mg/dL 80-120

PATHOLOGY CENTRAL

TEST	RESULT	NORMAL RANGE
COMPLETE BLOOD COUNT (CBC)		
Hemoglobin	10.6 gm/dl	12-18
Lympho	16.4 %	20-40
Mono	6.5 %	2-10
Eosin	2.3 %	1-6
Baso	0.3 %	0-1
TREC	4.14 10E6/	4.5-5.5
	UL	
PLATELET COUNT	375 10 ³ /U	150-400
	L	
HCT	32.2 %	40-50
MCV	77.6 fl	83-101
Neutro	72.7 %	40-70
MCH	25.5 pg	27-32
MCHC	32.9 g/dl	32-37
TOTAL WBC	7.33 10 ³ /	4-11
	UL	
MPV	8.4 fl	7-9
PCT	0.31 %	0.15-0.32
RDW-CV	14.7 %	11.5-14.5

Signature

SAWAI MAN SINGH HOSPITAL
SAWAI RAM SINGH ROAD SAWAI RAM SINGH ROAD 302004.
PATIENT TEST RESULT SLIP

Patient ID : 110325245330424
Patient Name: MR. Pawan Kumar Sharma Age/Gender : 43Y /M
Bill No : SMH/I/25/3984579 Barcode No : 548138091
OPD/IPD : A/SMH/25/112707
Report Date : 06-09-2025 06:30 PM
Bill Date : 06-09-2025
Department : GASTROINTESTINAL SURGERY
Category : NAA
Ward : 5 A WARD

IMMUNO HEPATO LAB

TEST	RESULT	NORMAL RANGE
Blood grouping (IH Lab) BLOOD GROUP	A	

BIOCHEM CENTRAL

TEST	RESULT	NORMAL RANGE
SERUM ALBUMIN	3.78 gm/dl	3.5-5.2
SERUM T.P./A G RATIO		
Total Protein	9.37 gm/dl	6.2-8.5
SERUM ALBUMIN	3.78 gm/dl	3.5-5.2
A:G RATIO	0.7	0.8-2
Globulin	5.59 g/dl	1.8 - 3.6
SERUM PHOSPHOROUS	5.14 mg/dl	Children - 4.0 - 7.0 Adult - 2.5 - 3.0
	8.22 mg/dl	8.8-10.2
SERUM CALCIUM	99.7 IU/L	New Born 95-36 8 Children(2-11y rs.) 115-403 Adult Male 41- 137 Adult Female 3 9-118
SERUM ALKALINE PHOSPHATASE		

SAWAI MAN SINGH HOSPITAL,
SAWAI RAM SINGH ROAD SAWAI RAM SINGH ROAD 302004,
PATIENT TEST RESULT SLIP

ID : 110325245330424
Patient Name: MR. Pawan Kumar Sharma Age/Gender : 43Y /M
Bill No : SMSH/1/25/3986579 Barcode No : 548138091
OPD/IPD : A/SMSH/25/112707
Report Date : 06-09-2025 08:06 PM
Bill Date : 06-09-2025
Department : GASTROINTESTINAL SURGERY
Category : PAAY
Ward : 5 A WARD

SEROLOGY

TEST	RESULT	NORMAL RANGE
------	--------	--------------

ANTI HCV (RAPID TEST)

ANTI-HCV

Negative ✓

Note:- Confirm by HCV RNA PCR

Remark:- ANTI HCV is a screening test of lower sensitivity and false negative results may be encountered with such tests.

HBs Ag (RAPID TEST)

HBsAg

Negative ✓

Note :- Confirm by HBV DNA PCR

Remark :- HBs Ag (Rapid Test) is a screening test of lower sensitivity and false negative results maybe encountered with such tests.

IMMUNO HEMATO LAB

TEST	RESULT	NORMAL RANGE
------	--------	--------------

Blood grouping (IH Lab)

BLOOD GROUP

A-

BIOCHEM CENTRAL

TEST	RESULT	NORMAL RANGE
------	--------	--------------

SERUM ALBUMIN

3.78 gm/dl 3.5-5.2

HIV Test Laboratory, Department of Microbiology SMS Medical College, Jaipur

LABORATORY REPORT FORM FOR HCTS SCREENING FACILITY (CONFIDENTIAL)

NAME OF HCTS FACILITY :

MICROBIOLOGY LABORATORY, DHANWANTRI OPD BLOCK

ADDRESS :

SMS HOSPITAL, JAIPUR

Name : Pawan kumar Age 43 Gender : ☒ M / F / TG

Ref. From : PID : GC FICTC/RJ/JPR/SMH/TC/20.....

*Blood (C.S./D.S.) : (DD/MM/YY) / (HH : MM)/Lab ID : 68361

Date/Time of Sample Collection : 6 SEP 2025 (DD/MM/YY) 2:00 (HH : MM)

Test Details

Sample : Serum / Plasma / Whole Blood

Sample tested on : 6 SEP 2025 (DD/MM/YY) 2:00 (HH : MM)

RESULTS OF HIV ANTIBODY TEST :

NON-REACTIVE

Name of HIV Test Kit u Batch No. :

*C.S.- Collected Sample Received

*D.S.- Sample Drawn at HIV Test Laboratory

Note : The report is of the sample received / drawn. The identity (Name, Age and Sex) of the client is a per information given by client/referring doctor/referring center.

LAB TECHNICIAN
MICROBIOLOGY LAB
SMS HOSPITAL
Name & Signature
Laboratory Technician

RESIDENT
MICROBIOLOGY LAB
SMS HOSPITAL
Name & Signature
Laboratory In-charge

— End of report —

DEPARTMENT OF RADIODIAGNOSIS
SMS MEDICAL COLLEGE & HOSPITAL, JAIPUR

SONOGRAPHY REPORT

Grossly Abdomen

Reg. No.

Date *8/6/25*

NAME *Pawan* Age Sex Indoor/

Outdoor USG Done By Dr.

Liver : Is normal is size, shape & echo texture

17.6 cm

Bright

altered

liver

echotexture

No focal or diffuse lesion is seen in liver parenchyma. IHBR not dilated.

CBDmm diameter. Spleno portal axis PVmm

SVmm

Gallbladder : is visualized with echo free lumen & normal wall thickness.

not visualised

no stone seen in GB fossa region

Pancreas : is normal in size & echotexture MPDmm.

observed

→ falciform seen in RIF in subcutaneous

minimally Hypoechoic collection noted

Kidneys : both kidneys are normal in size, shape & echotexture.

Corticomedullary differentiation is maintained, Size RK *WNL* LK *WNL*

Spleen : is normal in size & echotexture.

U.B. : Bladder wall smooth with normal thickness, Lumen is echo free.
Pre voiding volume.....ml. Post void Residual volume.....ml.

Free Fluid/Lymphadenopathy

Uterus

Size

Smooth contour

Homogeneous echotexture

Endometrial echoes are central normal thickness.

Ovaries

Size Rt Lt

Prostate

Normal in size & echotexture

Sizemm

Weightgms

Impression : *→ Liver Parenchymal disease*

Hepaticomegaly

Signature of Radiologist

DEPARTMENT OF GASTROENTEROLOGY

S.M.S MEDICAL COLLEGE AND HOSPITAL, JAIPUR.

DISCHARGE TICKET (GE III)

DR. GAURAV GUPTA

PROFESSOR AND UNIT HEAD

DR. HEMENDRA BHARADWAJ

PRINCIPAL SPECIALIST

DR. MUKESH JAIN

PROFESSOR

NAME: MR. PAWAN KUMAR SHARMA

AGE/SEX: 43/M

REG NO: A/SSH/25/12203

DOA: 28-6-25

DOD: 2-7-25

ADDRESS: Jhunjhunu, Rajasthan

**DIAGNOSIS: ACUTE PANCREATITIS- 3rd MONTH - BILIARY- SEVERE- PAST INFECTED
WON/LEFT PLEURAL EFFUSION / AKI (RESOLVED)/ ASCITES
ERCP + 2 DPT ADDED (1/07/2025)**

S/P PCD FOR PANCREATIC COLLECTION APRIL 2025

S/P OPEN PANCREATIC NECROSECTOMY (09/05/2025)

S/P CALCULOUS CHOLECYSTITIS - CHOLECYSTOSTOMY (09/04/2025)

COURSE IN WARD: Patient came to us with c/o high output from cholecystostomy drain. Patient diagnosed case of acute biliary pancreatitis for 3 month. Past h/o pancreatic necrosectomy i/v/o persistent sepsis despite PCD in WON. Patient is now referred from Dept of GI surgery for ERCP + DPT to reduce the drain output from CHOLECYSTOSTOMY. Alcohol- Past CRTS+, O2+, PCD+, Hemodialysis- Ascites +, left pleural effusion+. Past h/o Therapeutic pleural Tap 3 times in past. PF analysis showed elevated amylase

O/c- at admission: pallor-, icterus -, cyanosis-, clubbing- Edema- JVP NR

Patient was investigated with routine investigations, X-ray, ECG, USG W/A and ERCP.

Patient was appropriately investigated and managed conservatively and ERCP+ 2DPT PLACED ON 01/07/2025. now being discharge in vitally stable condition and advised to follow up with Department of Gastroenterology after 2 weeks.

**TREATMENT
GIVEN:**

Antibiotics

Laxatives

Pantop

IVF

Tramadol

LEVOSULPIRIDE

TREATMENT ADVISED: ALCOHOL ABSTINENCE

TAB FAROPENEM 200MG BD FOR 7 DAYS

TAB PANTOP 40MG OD

TAB ONDAN MD SOS FOR VOMITING

TAB TRAMADOL 1 SOS FOR PAIN

SYP PARAFFIN 15ML BD

X for 14 days

Senior Resident
Gastroenterology Deptt.
SMS Med College, Jaipur

DATE	26/6/25	17/25
Hb	11	9.9
TLC	8750	9550
DIFF %	61.25	69.20
Platelet	5.19	5.28
MCV	93	93
PT INR	1.21	1.56
RBS	81	81
Burea Creat.	130.5	130.6
S. Na k	130.47	134.44
S total Prot.	7.55	6.6
S albumin	3.4	3.1
Bilirubin (T.D)	0.7/0.4	0.6/0.3
SGOT	28	25
SGPT	28	26
SAP	163	157
GGT	81	39
S. Amylase	35	34
S lipase	45	260
LDH	407	8/4.2
Ca/PO4	9.2/4.1	
ESR- 50		
PCT - 0.12		
TG - 130		
Blood CS - Awaited		
Bile CS - Awaited		

HbsAg	NEG
Anti HCV	NEG
HIV	NEG

ERCP 01/07/2025
D1 - normal, D2 - AMPULLA IDENTIFIED - CBD IDENTIFIED. SELE
CANNULATION DONE. EST DONE. GW PLACED. 2 DPT 7TR PLACED
SYSTEM AND ONE IN RIGHT SYSTEM.
IMPRESSION:- ACUTE PANCREATITIS BILIARY S/P CHOLECYSTECTOMY
ERCP DONE TWO 7TR DPT PLACED IN CBD.

MRCP 12/06/2025

Moderate left pleural effusion. Free fluid is seen in peritoneal cavity - mild to moderate ascites. Drainage catheter is seen in right upper abdominal cavity. Collection seen in perinephric and paranephric around lower pole of left kidney extending into left iliac fossa. Collection measuring 115x90mm. Stranding seen around left posterolateral abdominal wall. Liver is normal in morphology and signal intensity. No focal parenchymal lesion is seen. IHBR are not dilated. Portal vein is normal. Stone is seen in GB. CBD is normal in course and caliber. No stone is seen. Pancreas is normal in size and signal intensity. Pancreatic duct is not dilated. Peripancreatic region is normal.

CT THORAX 06/06/2025

Area of collapse consolidation is seen involving bilateral lower lobes. Mild bilateral pleural effusion is seen with collapse consolidation of underlying lung parenchyma. Few subcentimeter sized pretracheal, paratracheal, prevascular, AP window, subcarinal and bilateral hilar lymph nodes noted.

IMPRESSION - Findings are suggestive of infective etiology, likely bacterial.

USG W/A 29/06/2025

Liver 16.4 cm, bright with altered echotexture. NO IHBRD. GB - contracted with few calculi largest measuring 12mm in GB fossa region. Pancreas - Heterogeneous with peripancreatic fat stranding. Left sided pleural effusion of depth 16mm at left CP angle, thick non aspirable. IMPRESSION - HEPATOMEGALY WITH LIVER PARENCHYMAL DISEASE WITH CHOLELITHIASIS WITH PANCREATITIS WITH LEFT PLEURAL EFFUSION.

UPPER GI ENDOSCOPY 30/06/2025

Esophagus - Normal
GE junction - LAX LES HILLS GRADE II
Stomach - Normal
Duodenum - Normal

IMPRESSION - LAX LES HILLS GRADE II

MRCP: dated 02-04-2025 - Bulky edematous pancreas with patchy areas of necrosis, marked peripancreatic inflammatory stranding, inflammatory thickening of bilateral Gerota's fascia and mild fluid of bilateral pararenal spaces suggestive of severe grade acute pancreatitis. Cholelithiasis with features of cholecystitis. Normal CBD with no intraductal calculus.

CECT whole abdomen: dated 08-04-2025 - Bulky edematous pancreas with approx 50% necrotic parenchyma, marked peripancreatic inflammatory stranding, mild peripancreatic fluid and inflammatory thickening of Gerota fascia and dilated jejunal loops suggestive of severe grade acute necrotizing pancreatitis (modified CT severity index = 10/10) Fatty liver Mild ascites and mild pleural effusion and diffuse abdominal wall edema. Gall bladder sludge.

CECT Chest: dated 08-04-2025 - Bilateral mild pleural effusion with basal lung atelectasis.

USG W/A: dated 30-04-2025 - 170-200cc collection seen in lesser sac. Moderate left effusion. Gall bladder collapsed. Cholecystostomy tube in situ.

CECT whole abdomen: dated 07-05-2025 - Large walled-off necrotic collection in pancreas extending in lesser sac with residual inflammatory changes in peripancreatic region in peripancreatic fat and inflammatory thickening of Gerota fascia residual changes of pancreatitis. Fatty liver. Right renal lower pole calculi. Marked inflammatory attenuation of SMV and proximal splenic vein. With perigastric collaterals. Borderline dilatation of proximal jejunal loops with no focal transition point likely localized ileus.

Follow Up:

- GE OPD after 2 week/ SOS
Wednesday / Saturday
Attach to GI surgery for Cholecystectomy.

Signature:

DR. JAYKISHAN SINGH NARWARIYA

Senior Resident
Gastroenterology Deptt.
SMS Med. College, Jaipur

ERCP REPORT
DEPARTMENT OF GASTROENTEROLOGY
SUPER SPECIALITY HOSPITAL, SMS MEDICAL COLLAGE, JAIPUR
UNIT-III

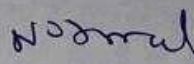
NAME/ ID	PAWAN KUMAR SHARMA/12208
AGE/SEX	48/M
DATE	1/07/2025
DIAGNOSIS	ACUTE PANCREATITIS – BILLIARY – S/P CHOLECYSTOSTOMY
PREMEDICATION	INJ- BUSCOPAN, INJ-MIDAZOAM, INJ-BUTORPHANOL,
- AMPULLA - CBD - MPD - PROCEDURE	IDENTIFIED. SELECTIVE CBD CANNULATION DONE. EST DONE. GW PLACED. 2 DPT 7FR PLACED ONE IN <u>LEFT</u> SYSTEM AND ONE IN <u>RIGHT</u> SYSTEM. ERCP DONE – <u>TWO 7 FR DPT PLACED</u>
IMPRESSION:-	ACUTE PANCREATITIS – BILLIARY – S/P CHOLECYSTOSTOMY ERCP DONE – TWO 7 FR DPT PLACED IN CBD
ADVICE:-	NPO FOR 6 HOURS IV ANTIBIOTICS IV FLUIDS
DOCTORS	Dr. Gaurav Gupta Dr. Mukesh Jain Dr. Hemendra Bharadwaj

डॉ. मुकेश जैन
 गुरुदेव गुरुद्वारा
 सिद्धि प्राप्तये नमः

GASTROSCOPY REPORT

Name Pawan Kumar Age/Sex Date 30/6/20

Diagnosis.....



INTERPRETATION

low LES will go II

①2 sahil

ENDOSCOPIST

ENDOSCOPIST
डॉ. विद्याधर सिंह निरंतर महा आसि
डॉ. रेखा व्यास ज्य निदेशक

SUPER SPECIALITY HOSPITAL

SMS Medical College, Jaipur

Department of Radiodiagnosis

SONOGRAPHY REPORT

HID No. Date 29/6/25
 Name Pawan Kumar Age 46/10 Sex Male Indoor/Outdoor Indoor

Liver : Is normal in size, shape & echo texture 16.4 cm Bright, altered. No IHBR
 No focal or diffuse lesion is seen in liver parenchyma. IHBR are not dilated.
 CBD (N) mm PV mm SV mm

Gallbladder : is visualized with echo free lumen & normal wall thickness. contracted to low volume 1 cm in GB lumen peripancreatic heterogeneity

Pancreas : is normal in size & echo texture MPD mm. Heterogeneous

Kidneys : both kidneys are normal in size, shape & echotexture.
 Corticomedullary differentiation is maintained, Size RK cm mm LK cm mm.
 Cortical thickness.

Spleen : is normal in size & echotexture. 1 cm enlarged

U.B. : Bladder wall is smooth and normal in thickness, Lumen is echo free.
 Pre voiding ml. Post void Residual ml.

Free Fluid/Lymphadenopathy

Uterus
 Size
 Smooth contour
 Homogeneous echotexture
 Endometrial echoes are central normal thickness.

Prostate * (L) sided thick
 Normal in size & echotexture
 Size mm
 Weight gms

Ovaries

Size Rt Lt

Impression :

① Hepatic injury
 liver parenchymal disease
 ② cholelithiasis
 ③ pancreatitis
 ④ (L) sided thick
 effusion of
 depth 16mm at
 post 11 o'clock. Post 12 o'clock
 held clear
 Signature of Radiologist

NOT VALID FOR MEDICO LEGAL PURPOSE

HIV Test Laboratory, Department of Microbiology SMS Medical College, Jaipur

LABORATORY REPORT FORM FOR HCTS SCREENING FACILITY (CONFIDENTIAL)

NAME OF HCTS FACILITY : MICROBIOLOGY LABORATORY, DHANWANTRI OPD BLOCK
ADDRESS : SMS HOSPITAL, JAIPUR

Name : Pawan Age 43 Gender M / F / TG

Ref. From : PID : GC FICTC/RJ/JPR/SMSH/TC/20.....

*Blood (C.S./D.S.) : (DD/MM/YY) / (HH : MM) / Lab ID : 48353
Date/Time of Sample Collection : 28 JUN 2025 (DD/MM/YY) 2:00 (HH : MM)

Test Details

Sample : Serum / Plasma / Whole Blood

Sample tested on : 28 JUN 2025 (DD/MM/YY) 8:00 (HH : MM)

RESULTS OF HIV ANTIBODY TEST :

NON-REACTIVE

Name of HIV Test Kit ms Batch No. :

*C.S.- Collected Sample Received

*D.S.- Sample Drawn at HIV Test Laboratory

Note : The report is of the sample received / drawn. The identity (Name, Age and Sex) of the client is a per information given by client/referring doctor/referring center.

LAB TECHNICIAN
Name & Signature
MICROBIOLOGY LAB
Laboratory Technician
SMS HOSPITAL

RESIDENT
Name & Signature
MICROBIOLOGY LAB
Laboratory In-charge
SMS HOSPITAL

— End of report —

SAWAI MAN SINGH HOSPITAL JAIPUR

SAWAI RAM SINGH ROAD TONK ROAD

LAB REPORT

Patient's Name : MR. Pawan Kumar Sharma
Age/Gender : 43Y /M
Bill Id :-
Bill No : SSMH/1/25/3046556
Department : GASTROINTESTINAL SURGERY

HID : 110325245330424
Bill Date : 12-07-2025
Patient Category : DOMICILE OF RAJASTHAN
Report Date/Time : 12-07-2025 10:04:35 AM
Opd/lpd No : V/SMH/25/1714628

Test Name/Parameter	Result/Notes	Unit	Reference Range
BIOCHEMISTRY Report			
SERUM UREA	12.2	mg/dL	10-50
SERUM Creatinine	0.63	mg/dL	Male 0.6-1.5 Female 0.6-1.4
Electrolyte			
Na	133.4	mmol/L	135-145
K	5.00	mmol/L	3.5-5.5
Cl	95.7	mmol/L	95-105
SERUM ALBUMIN	3.02	gm/dl	3.5-5.2
SERUM BILIRUBIN			
Total Bilirubin	0.7	mg/dL	Up To 1.1
Direct Bilirubin	0.3	mg/dL	Up To 0.25
Indirect Bilirubin	0.40	mg/dL	0.2-1.0
SGOT	14.7	U/L	0-35
SGPT	12.9	U/L	0-40
SERUM ALKALINE PHOSPHATASE	401.4	IU/L	New Born 95-368 Children(2-11yrs.) 115-403 Adult Male 41-137 Adult Female 39-118

Hematology Report

COMPLETE BLOOD COUNT (CBC)			
Hemoglobin	10.8	gm/dl	12-18
TOTAL WBC	13.40	10 ³ /UL	4-11
TRBC	3.96	10E6/UL	4.5-5.5
MCV	88.9	fl	83-101
MCH	27.3	pg	27-32
MCHC	30.7	g/dl	32-37
RDW-CV	21.4	%	11.5-14.5
PLATELET COUNT	707	10 ³ /UL	150-400
HCT	35.2	%	40-50
PCT	0.66	%	0.15-0.62
MPV	9.3	fl	7-9
Neutro	68.5	%	40-70
Lympho	21.4	%	20-40
Mono	8.4	%	2-10
Eosin	1.4	%	1-6
Baso	0.3	%	0-1
MCV	88.9	fl	83-101
Prothrombin Time (PT)			
INR	1.27	ratio	-
	16.4	sec	10-15



S.M.S. HOSPITAL, JAIPUR

SONI HOSPITALS CT & MRI CENTRE

Patient Name	PAWAN KUMAR SHARMA	Patient ID	532014146
Age	43Yrs.	Date	31-Jul-25
Gender		Ref. Doctor	SMS

TRIPLE PHASE CECT SCAN WHOLE ABDOMEN

Clinical history is not available.

Pancreas appear diffusely bulky and edematous with hypodense attenuation as well as perinephric fluid and fat stranding which is extending into the perirenal region in the form of lateral and anterior conal fascia thickening as well as associated thickening of colon and gastric bowel loops however no obvious pseudo cyst formation seen – above finding likely to be acute interstitial pancreatitis.

Biliary stent in situ with pneumobilia.

Gall bladder is not visualized with drainage tube in situ in gall bladder fossa region.

Mild interloop ascites is seen.

Minimal left pleural effusion is seen with underlying compressive atelectasis.

Liver is normal in size and has smooth borders. Hepatic parenchyma shows normal attenuation.

Spleen and both adrenals are normal in size and shape.

Both kidneys are normal in anatomical position and structure.

Urinary bladder is distended and has smooth walls.

Pelvic organs appear normal.

Adv:- Clinical correlation.

Dr. Mukesh Kr. Sharma, M.D, DNB

Consultant Radiologist (DMC No. 25362/13974)

Any clinical discrepancy please discuss with radiologist. Patient's identification in reporting is not established, so in no way this report can be utilized for any medico-legal purpose.

(Disclaimer: This report is provisional and needs medical history, it may be completely altered after receipt of the prior medical history of the patient).

• This report is not valid for medico-legal purpose • All subject to Jaipur jurisdiction only.



S.M.S. HOSPITAL, JAIPUR

SONI HOSPITALS CT & MRI CENTRE

Patient ID :	513162706	Patient Name :	Pawan Kumar
Age :	43 Years	Sex :	M
Accession Number :	SCMC2025069939	Modality :	MR
Ref. Physician :	Dr. RAM DAGA	Study :	MRCP
Study Date :	12-Jun-2025		

Findings :

MRCP

Moderate left pleural effusion.

Free fluid is seen in peritoneal cavity – mild to moderate ascites.

Drainage catheter is seen in right upper abdominal cavity.

Collection is seen in perinephric and paranephric around lower pole of left kidney extending into left iliac fossa, collection measuring 115x90mm. stranding seen around left posteriolateral abdominal wall

Liver is normal in morphology and signal intensity. No focal parenchymal lesion is seen. IHBR are not dilated. Portal vein is normal.

Stone is seen in GB.

CBD is normal in course and caliber. No stone is seen.

Pancreas is normal in size and signal intensity. Pancreatic duct is not dilated. Peripancreatic region is normal.

Spleen is normal in size and morphology.

Both adrenal glands are normal.

Right kidney is normal.

No adenopathy is seen.

Visualized bowel loops are normal.

Dr. Rakesh Lal, M.D.

RMC: 17076

Consultant Radiologist

• This report is not valid for medico-legal purpose • All subject to Jaipur jurisdiction only.

DEPARTMENT OF RADIODIAGNOSIS
SMS MEDICAL COLLEGE & HOSPITAL, JAIPUR

SONOGRAPHY REPORT

Reg. No.

F/U/C of Acute necrotising
pancreatitis

Date 17/6/25

NAME Panman Age Sex Indoor/

Outdoor USG Done By Dr.

Liver : Is normal is size, shape & echo texture

17.4 cm, Bright altered echotexture
No focal or diffuse lesion is seen in liver parenchyma. IHBR not dilated.
CBDmm diameter. Spleno portal axis PVmm
SVmm

Gallbladder : is visualized with echo free lumen & normal wall thickness.

N/V → ? Contracted

→ Duct tip seen in GB fossa region
→ collection of 20mm depth noted in GB fossa region

Pancreas : is normal in size & echotexture MPDmm.

→ visualised Part of Pancreas (Head & Body) appears heterogeneous, Bulky, Peri pancreatic fat

Kidneys : both kidneys are normal in size, shape & echotexture.

Corticomedullary differentiation is maintained, Size RK WNL LK WNL fat sparing

Spleen ✓ : is normal in size & echotexture.

→ few echogenic foci noted in pancreatic region

U.B. : Bladder wall smooth with normal thickness, Lumen is echo free.
minimally filled
Pre voiding volumeml. Post void Residual volumeml.

Free Fluid/Lymphadenopathy

Uterus

Size
Smooth contour
Homogeneous echotexture
Endometrial echoes are central normal thickness.

Prostate ✓

Normal in size & echotexture
Sizemm
Weightgms

Ovaries

Size Rt. Lt.

Impression :

→ Liver Parenchymal disease
→ collection as describe above
→ Acute necrotising Pancreatitis

Signature of Radiologist

Dr. Jyoti
Dr. Pradip

DEPARTMENT OF SURGICAL GASTROENTEROLOGY,



MINIMAL ACCESS & BARIATRIC SURGERY

Santokba Durlabhji Memorial Hospital cum Medical Research Institute,
Bhawani Singh Marg, Jalpur - 302015 Ph. +91 141 2560251 - 58 (Ext. 111 & 423)



DISCHARGE ON REQUEST AGAINST MEDICAL ADVICE

Name	Mr. Pawan Kumar Sharma	Age/Sex	43yrs/Male
UHID No.	20593076	IPD No.	616039
Address	Ward No. 16, Sujargarh, Jhunjhunu Ph. 9772015438		
Diagnosis	Moderately severe acute necrotizing pancreatitis with infected pancreatic necrosis		
Procedure	Open <u>necrosectomy</u> with lavage and drainage		
ICD Code	K-85		
Date of Admission	29-04-2025		
Date of Operation	09-05-2025 ✓		
Date of Discharge	23-05-2025 ✓		
Unit	GI Surgery (Dr. Rajesh Bhojwani/Dr. Saurabh Jain/ Dr. Abadhesh Sharma)		

History :
C/o

Follow up case of severe acute necrotizing biliary pancreatitis with calculus cholecystitis since April 2025.
History of USG guided percutaneous cholecystostomy on 09-04-2025 ✓
History of USG guided percutaneous drainage on 15-04-2025 ✓
History of CT guided percutaneous drainage on 15-04-2025 ✓
Now admitted with complaints of fever for 5-7 days
Pain in epigastric region for 2-3 days
Not associated with nausea and vomiting
No history of obstipation and constipation
No history of jaundice, pruritis and weight loss
No history of haemetemesis, haematuria & malena
No history suggestive of DM, HTN, TB & Drug allergy
No other morbid risk factors

Examination :

P/A Mild distension +
Bowel sounds normal
No tenderness in upper abdomen
Cholecystostomy tube in situ
PCD in situ

Other investigations :

CHEST X RAY - Normal Study

Special investigations:

MRCP: dated 02-04-2025 ?

Bulky edematous pancreas with patchy areas of necrosis, marked peripancreatic inflammatory stranding, inflammatory thickening of bilateral Gerota's fascia and mild fluid of bilateral pararenal spaces suggestive of severe grade acute pancreatitis.
Cholelithiasis with features of cholecystitis.
Normal CBD with no intraductal calculus.

CECT whole abdomen: dated 08-04-2025

Bulky edematous pancreas with approx 50% necrotic parenchyma, marked peripancreatic inflammatory stranding, mild peripancreatic fluid and inflammatory thickening of Gerota fascia and dilated jejunal loops suggestive of severe grade acute necrotizing pancreatitis (modified CT severity index = 10/10).
Fatty liver.
Mild ascites and mild pleural effusion and diffuse abdominal wall edema.
Gall bladder sludge.

CECT Chest: dated 08-04-2025

Bilateral mild pleural effusion with basal lung atelectasis.

30/4
2/13
USG Screening: dated 30-04-2025

170-200cc collection seen in lesser sac. Moderate left effusion.
Gall bladder collapsed. Cholecystostomy tube in situ.

USG Screening: dated 03-05-2025

No abdominal collection seen. Minimal ascites.
Left moderate effusion.

CECT whole abdomen: dated 07-05-2025

Large walled-off necrotic collection in pancreas extending in lesser sac with residual inflammatory changes in peripancreatic region in peripancreatic fat and inflammatory thickening of Gerota fascia residual changes of pancreatitis.

Fatty liver.

Marked inflammatory attenuation of SMV and proximal splenic vein with perigastric collaterals.

Right renal lower pole calculi.

Borderline dilatation of proximal jejunal loops with no focal transition point likely localized ileus.

Procedure:

Open necrosectomy with lavage and drainage

Operative findings:

Omental saponification and necrosis present

No gross intra-abdominal collection

Small bowel and colon grossly normal

Tense, bulge noted in mesocolon

Lesser sac opened via Transmesocolic route

Evidence of necrosis including pancreas body and tail extending to left pararenal space, roughly ~ 300gm necrosum removed with around 1 liter purulent collection.

Lavage done

Interdepartmental consultation:

Dr. Lalit Garg (Interventional Radiologist)

Dr. M.L. Gupta (Pulmonologist)

Hospital Course:

Follow up case of severe acute necrotizing biliary pancreatitis with calculus cholecystitis status percutaneous cholecystostomy and PCD insertion, admitted with above mentioned complaints in intensive care unit. On screening USG abdomen done on 30/04/25 he was found to have ~ 170-200 cc collection in lesser sac with moderate left sided pleural effusion, for which USG guided PCD insertion and therapeutic left sided effusion tapping (~500 ml) done on 30/04/25. After stabilization of general condition he was shifted to ward and had persistent O2 requirement. His antibiotics were upgraded according to culture sensitivity reports. For recurrent development of pleural effusion despite of intensive spirometry and chest physiotherapy, USG guided left side pleural tapping was done on 03/05/25 (~750ml). In view of persistent fever and necrotic PCD contents CECT whole abdomen done on 07/05/2025 was suggestive of large walled-off necrotic collection in pancreas extending in lesser sac with residual inflammatory changes in peripancreatic region in peripancreatic fat and inflammatory thickening of Gerota fascia residual changes of pancreatitis. He underwent open necrosectomy with lavage and drainage done on 09/05/2025. Postoperatively he was managed in intensive care unit with supportive treatment and was shifted to ward on 3rd postoperative day starting him on oral liquids. He underwent USG guided left side pleural effusion tapping on 13/05/2025 (~500ml). His fever gradually subsided and he improved symptomatically. Attendants of the patient were explained regarding need for further in hospital treatment and observation, but they are insisting on discharge, hence patient is discharged on request against medical advice. Risk of transportation explained to relatives.

Condition on discharge:

On normal diet, passing flatus and motion, ambulatory

Advise on discharge:

Diet: Normal (Fat free, high protein)

Activity: Normal

Dressing: Not required

Sutures: None

Cholecystostomy tube in situ (care and daily output monitoring)

Lesser sac and transmesocolic drain tubes in situ (care and daily output charting)

IV Cannula in situ (care of cannula)

Treatment: Inj. Colistin 3 MIU IV TDS for 7 days सुबह-दोपहर-रात

• Fosfomycin sachet 3gm EOD for 7 days - एक दिन दो बार

• Tab. Forcan 200mg 1 BD for 7 days सुबह-रात

• Tab. Pantocid 40mg 1 once a day (before meal) AC for 07 days सुबह खाने की 1 घंटा पहले

• Tab. Supradyn 1 OD for 15 days दिन में 1 बार

• Tab. Crocin 650mg 1 SOS दुखे/बुझावे

Tab. Enzar-HS 1 TDS (during meal) to be continued सुबह-दोपहर-रात

Repeat Serum creatinine, CBC every 3rd day and inform

Intensive spirometry and chest physiotherapy as advised 5/06/2025

Review in SOPD with prior appointment on Thursday on 01-06-2025

or SOS between 10:00 AM to 2:00 PM (Room No. 208)

(Dr. Rajesh Bhojwari)

Helpline: In case of fever, vomiting, excessive pain in abdomen, distension of abdomen, fainting, excessive discharge through the operation site or any bleeding from mouth/anus, please contact at 8890005999.

For appointments: 8890005999

19/ Anil Kumar Sharma
Anil Poonacha

Signature and date Patient/Relatives
(Discharge received)

Department of Pathology and Bacteriology, S.M.S. Hospital, Jaipur
Requisition for Surgical Pathology Material

Name..... ~~P. P. Patel~~ **PAWAN** Age..... Sex.....

Path No..... ~~0004210842041~~

M.O. Incharge..... Ward/Bed.....

Regd. No..... 110328245330424

Gross Description of Specimen:-
 A. AC Biliary pancreas
 Cholelithiasis & cholecystitis

(P20): LAP. Cholecystectomy

(H): GB wall thickened &
 filled w/ large stone - 2

Microscopic description and/or Diagnosis :-

Spec sent for H&E

① GB wall

Dr. ANA VANGID
 Resident
 Surgery
 Jaipur

Date.....

DEMONSTRATOR



MUKHYAMANTRI AYUSHMAN AROGYA YOJANA

TID: T06092541882749
Case Type: Normal
Hospital Name: Sawai Man Singh Hospital
Identity No.: 5162263555
Discharge Date: 2025-2026
Policy Year: 06-Sep-25 02:32 PM
Creation Date: 06-Sep-25 02:32 PM
IT services Failure:
Claim Submission:
HHID:
Jan Aadhar id: 5162263555
Beneficiary Enroll Mob No: 7849966705
RTA Flag: No
Patient Name: Pawan Kumar Sharma(पवन कुमार शर्मा)/M
Identity Type: Jan Aadhar
Admission Date: 06-Sep-25 11:22 AM
Hospital Type: Government
Reference Tid:
Admission No: 88520609259415146
Enrol Verify By: Pawan Kumar Sharma
Administrative Error:
Entitlement Type: Paid
Registered Mob No: 9887313392
Discharge Mob No: None

Package Code	Package Category	Package Rate (Rs.)	Serial Number	Package Status	Package Remarks
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5162263555	GO. Secondary Health	10000	10000	Not Authorized	
5162263555	Original				

Package Code Approved Amount TDS Amount: Net Approved Amount: Paid Amount